

1. Administrative & Study Identification

Full Study Title: A Phase III, Multicenter, Randomized, Open-Label Study Comparing the Efficacy and Safety of Glofitamab in Combination With Polatuzumab Vedotin Plus Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone (Pola-R-CHP) Versus Pola-R-CHP in Previously Untreated Patients With Large B-Cell Lymphoma **Short Title/Acronym:** SKYGLO **Protocol Number:** G044145 **NCT Number:** NCT06047080 **Posting ID:** 310350774548791924 **Sponsor / Company:** Hoffmann-La Roche / Roche **Investigational Product:** Glofitamab in combination with Polatuzumab Vedotin, Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone (Pola-R-CHP) **Phase of Development:** Phase III **Trial Status:** RECRUITING **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of glofitamab in combination with Pola-R-CHP compared with Pola-R-CHP alone in previously untreated patients, primarily measured by Progression-Free Survival (PFS) as determined by an Independent Review Facility (IRF). **Secondary Objectives:** To evaluate Overall Survival (OS), PFS in patients with IPI 3-5 (investigator and IRF assessed), Event-free survival efficacy causes (EFSeff), safety, pharmacokinetics (PK), immunogenicity, and health-related quality of life (HRQoL).

2.2 Background & Rationale Diffuse large B-cell lymphoma (DLBCL) is the most common subtype of aggressive non-Hodgkin lymphoma. While Pola-R-CHP recently became an approved standard first-line therapy demonstrating a PFS benefit over traditional R-CHOP, a significant portion of patients still do not achieve durable remissions. Glofitamab is a CD20xCD3 T-cell engaging bispecific antibody designed to bring T cells in close proximity to malignant B cells. This study aims to evaluate whether adding glofitamab to Pola-R-CHP improves clinical outcomes in previously untreated CD20-positive large B-cell lymphoma (LBCL) patients, aiming to elevate treatment standards and prevent relapse in the earlier stages of the disease.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Pola-R-CHP alone) **Blinding:** Open-label **Randomization:** Randomized (Arm A: Glofitamab + Pola-R-CHP vs. Arm B: Pola-R-CHP)

3.2 Planned Enrollment & Duration Targeted Enrollment: 1,130 participants **Disease Area:** Large B-Cell Lymphoma **Number of Sites:** Approximately 260 sites globally **Estimated Study Start:** 18-Sep-2023 **Estimated Primary Completion:** 01-Oct-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Previously untreated participants with CD20-positive large B-cell lymphoma (LBCL). 2. International Prognostic Index (IPI) score of 2-5. 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0, 1, or 2. 4. At least one bi-dimensionally measurable lesion, defined as > 1.5 cm in its longest dimension as measured by CT or MRI. 5. Ability to provide tumor tissue. 6. Left ventricular ejection fraction (LVEF) $\geq$ 50% on cardiac multiple-gated acquisition (MUGA) scan or cardiac echocardiogram (ECHO). 7. Adequate hematologic function. 8. Negative HIV test at screening with exceptions as defined by the protocol. 9. Negative SARS-CoV-2 antigen or PCR test.

4.2 Exclusion Criteria 1. Contraindication to any of the individual components of Pola-R-CHP or glofitamab, including prior receipt of anthracyclines, or history of severe allergic or anaphylactic reactions to humanized or murine monoclonal antibodies. 2. Prior solid organ transplantation. 3. Use of systemic immunosuppressive agents (e.g., cyclosporin, azathioprine, methotrexate, thalidomide, anti-TNF agents) within 4 weeks prior to first dose, or active autoimmune disease requiring treatment. 4. Current Grade > 1 peripheral neuropathy by clinical examination or demyelinating form of Charcot-Marie-Tooth disease. 5. History of indolent lymphoma or current diagnosis of: Follicular lymphoma grade 3B; transformations of indolent B-cell lymphomas; mediastinal grey zone lymphoma; primary mediastinal (thymic) LBCL; Burkitt lymphoma; primary LBCL of immune-privileged sites; primary effusion DLBCL; and primary cutaneous DLBCL, leg type. 6. Primary or secondary CNS lymphoma at the time of recruitment, or history of CNS lymphoma. Current or past history of CNS disease (e.g., stroke, epilepsy, CNS vasculitis, neurodegenerative disease), or history of progressive multifocal leukoencephalopathy (PML). 7. Prior treatment with systemic immunotherapeutic agents, use of any monoclonal antibody for cancer within 3 months, any investigational cancer therapy within 28 days, prior radiotherapy to the mediastinal/pericardial region, or prior therapy for LBCL (with the exception of corticosteroids). 8. Corticosteroid use > 30 mg/day of prednisone or equivalent, for purposes other than lymphoma symptom control. 9. History of other malignant or non-malignant diseases affecting compliance/interpretation; significant or extensive history of cardiovascular disease; clinically significant liver disease; recent major surgery (within 4 weeks), other than for diagnosis. 10. Known or suspected chronic active Epstein-Barr viral infection or history of hemophagocytic lymphohistiocytosis (HLH).

11. Positive test results for chronic hepatitis B infection, hepatitis C, or the human T-lymphotropic virus type 1 (HTLV-1); suspected active or latent tuberculosis. 12. Live, attenuated vaccine within 4 weeks before study treatment infusion on Day 1 of Cycle 1, or anticipation of needing one during the study. 13. Any active infection within 7 days prior to Cycle 1 Day 1 that would impact participant safety.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Treatment is administered in 21-day cycles for a total of 8 cycles. Arm A receives Glofitamab (with step-up dosing to mitigate side effects in Cycle 2, and a target dose in Cycles 3-8) alongside Pola-R-CHP in Cycles 1-6. Arm B receives Pola-R-CHP in Cycles 1-6, followed by Rituximab alone in Cycles 7 and 8. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** 8 cycles (approximately 24 weeks or 6 months).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications per institutional guidelines. **Prohibited:** Systemic immunosuppressive agents within 4 weeks prior to the first dose, and live attenuated vaccines during the study and until participants' B cells recover.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Informed Consent, Medical History, Tumor biopsy for CD20, baseline scans (CT/MRI), ECHO/MUGA
2-9	Cycles 1-8 (21 days/cycle)	IV administration of investigational products, glofitamab step-up dosing (Cycle 2), safety & efficacy assessments
10	Week 24	Final efficacy assessment, HRQoL evaluation, safety labs
11-18	Up to 5 years	Scans, telephone calls, and questionnaires every 3 to 6 months for survival and disease progression

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-free survival (PFS) as determined by an Independent Review Facility (IRF). **Measurement Method:** Time from randomization to the first occurrence of disease progression, relapse, or death from any cause.

7.2 Secondary & Exploratory Endpoints * Progression-free survival (PFS) as determined by the investigator. * PFS as determined by the investigator and IRF for participants with international prognostic index (IPI) 3-5. * Event-free survival efficacy causes

(EFSeff). * Overall Survival (OS) and Complete Response (CR) rate. * Pharmacokinetics (PK) profile and incidence of anti-drug antibodies (immunogenicity). * Health-related quality of life (HRQoL) and identification of predictive biomarkers.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Incidence and severity of AEs graded per NCI CTCAE v5.0. Specific monitoring for Cytokine Release Syndrome (CRS) graded per ASTCT criteria and neurotoxicity. **Serious Adverse Events (SAEs):** Evaluated continuously throughout treatment and up to 5 years of follow-up. **Data Monitoring Committee (DMC):** Internal/External safety monitoring mechanisms managed by the sponsor for phase III oncology trials.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy analysis. Safety Set for AE/SAE evaluations. **Sample Size Justification:** Approximately 1,130 participants adequately powered to detect a statistically significant shift in PFS when adding glofitamab to the Pola-R-CHP backbone. **Handling of Missing Data:** Standard oncology statistical methods for time-to-event endpoints (e.g., Kaplan-Meier methodology, Cox proportional hazards models, right-censoring for loss to follow-up).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of clinical sites to ensure protocol and regulatory compliance. **Audit Trail:** Validated electronic data capture (EDC) systems utilized with strict audit trails for all clinical data entries and imaging review via the IRF.

11. Study Identifiers & Governance

Primary ID: G044145 **Registry Number:** NCT06047080 **Sponsor/Lead Organization:** Roche / Hoffmann-La Roche **Study Title:** SKYGLO **Official Regulatory Title:** A Phase III, Multicenter, Randomized, Open-Label Study Comparing the Efficacy and Safety of Glofitamab (R07082859) in Combination With Polatuzumab Vedotin Plus Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone (Pola-R-CHP) Versus Pola-R-CHP in Previously Untreated Patients With Large B-Cell Lymphoma

12. Clinical Framework (Lymphoma Specific)

Primary Condition: Large B-Cell Lymphoma (LBCL) **Diagnosis Threshold:** CD20-positive, IPI score 2-5, ≥ 1 measurable lesion > 1.5 cm **Medical Methodology:** Chemoimmunotherapy combined with T-cell engaging bispecific antibody **Optimization Target:** Target dose of Glofitamab with Pola-R-CHP to achieve Complete Remission

13. Study Procedures & Methodology

Management Pathway: 21-day treatment cycles for 8 cycles **Education Protocol (HCP):** CRS management and step-up dosing training per site **Education Protocol (Patient):** Infusion schedules and AE reporting education **Quality Audit Mechanism:** Independent Review Facility (IRF) assessment and survival follow-up

14. Observation & Follow-Up Schedule

Total Duration: Up to 65 months (approx. 260 weeks) **Onsite Visit Cadence:** Every 21 days during the 24-week treatment period **Remote Monitoring Frequency:** Every 3 to 6 months during the follow-up period **Checklist Review Interval:** End of each 21-day cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				